



Fiona Stanley Fremantle Hospitals Group

OFFICIAL

Policy and Procedure

First on call: Mental Health Service

Reference #: FSFHG-MENH-PRO-0007

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital	Mental Health Service	Medical, Nursing, Allied Health, Administration

1. Introduction

This policy and procedure outlines the role of the First On Call (FOC) clinician and the processes required to ensure that active/new mental health consumers can access advice, treatment and care when the allocated Care Coordinator or Medical Practitioner are unavailable or when either of these roles have not been allocated. The role of the FOC is applicable to adult, older adult and youth community mental health services.

2. Terminology

First on call (FOC)	Appropriately qualified and experienced senior mental health professional or clinician under the supervision of a senior mental health professional who provides the first point of contact for the treatment team.
Active consumer	A person who has been assessed and accepted by a community treatment team for ongoing management of their mental health issues.
Care Coordinator	An identified Clinician from the Mental Health Service who has the responsibility for coordinating, facilitating and integrating the mental health treatment, care and support of an individual mental health consumer and their family/significant others.

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The information provided in this document is based on its relevance to Fiona Stanley Fremantle Hospital Group (FSFHG) only. Due to differences in context, scope of practice and differences in service delivery, FSFHG does not make claim to its relevance or appropriateness for use within other organisations/sites.

Youth Community Assessment and Treatment Team (YCATT)	Provides assessment and treatment for people aged 16 - 24 years who are experiencing current mental health symptoms or at increased risk of developing serious mental health issues.
Community Treatment Team (CTT)	Multidisciplinary team that provides services in the community where the consumer lives. This team works with the consumer and their family/significant others to monitor the consumer's wellbeing, progress on their treatment plan, identify risks and support recovery.
No access visit	Attempted home/community visit where the clinician has been unable to contact the consumer.
Intake	The multidisciplinary process for determining the provision of treatment required for a consumer which occurs at the next meeting following referral/review.
'No wrong door'	Access philosophy underpinning Mental Health Services which enables all referrals, regardless of presenting issues, to be considered by the multidisciplinary team and either accepted or redirected to the appropriate service as clinically indicated.
Psychiatric Services Online Information Service (PSOLIS)	Mental health clinical information system used by all government mental health agencies throughout Western Australia.

3. Policy

- The CTTs are responsible for responding to contacts from their active mental health consumers during normal business hours. For CTT business hours refer to Appendix 1.
- The CTT Team Leader/ Clinical Nurse Specialist (CNS) is responsible for:
 - creating a FOC monthly roster for their team, showing the discipline and clinician allocated as the FOC.
 - ensuring the FOC roster is made available via the Team Secretary and is accessible to all members of the team.
 - providing appropriate orientation and support to staff in the role of FOC
- The FOC will be available to respond to team clinical duties whilst on site. However, if FOC is going off site, the FOC responsibility must be delegated to another clinician on site who will attend to the FOC duties.

- The FOC will ensure contact is made with the consumer within two hours, or sooner if clinically indicated.
- All referrals will be triaged in line with the SMHS Triage assessment of referrals in community mental health.
- All contact with consumers will be person centred, trauma informed, culturally competent and developmentally appropriate.

4. Procedure

4.1. Administration staff

- Administration staff will initially:
 - assist with identifying the consumers registration details with the consumer.
 - and/or search webPSOLIS/PSOLIS
 - and/or check the checks next appointment in webPAS.
- Administration staff are not responsible for assessing the urgency of a presentation/call.
- Will inform the consumer's Care Coordinator/Team Leader/Senior Clinician/FOC of all presentations/calls.

4.2. Fremantle Hospital (FH)

- FH active community consumers who make phone contact with Triage Reception/main switchboard will have their phone call transferred to the respective Team Secretary who will redirect the call to the allocated Care Coordinator
- In the absence of the Care Coordinator/Medical Practitioner the phone call will be redirected to the FOC as per the FOC roster.
- If an active mental health consumer presents to the FH Triage Reception, they will be redirected to the Outpatient reception on Level 5
- Outpatient reception staff will inform the CTT Team Secretary of the consumers presentation.
- The Team Secretary will contact the consumers allocated Care Coordinator and inform them of the consumers presentation.
- If the Care Coordinator is unavailable and the consumer requires review the Team Secretary will inform the FOC

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- The FOC will:
 - Ascertain the reason for the consumer's presentation/call.
 - Check with the consumer or via webPAS to establish when the consumers next appointment is scheduled and based on the reason for the consumers presentation/call determine if the consumer is:
 - Able to wait until this appointment.

OR

 - Able to wait until the Care Coordinator/Medical Practitioner can return the consumers call.

OR

 - In need of immediate assistance
- In the absence of the FOC a backup clinician/Team Leader will be available to attend to the consumer.

4.3. Fiona Stanley Hospital (FSH)

- The YCATT FOC will:
 - perform initial triage on all new referrals.
 - answer general queries from referring agencies, external care providers and support persons.
 - ensures 'no wrong door' access.
 - take calls from active consumers when the Care Coordinator is unavailable and initiate the appropriate level of intervention.
 - provide clinical advice and support e.g. staff on home visits, no access visits
 - coordinate response to crises and/or psychological emergencies received by phone
- The YCATT FOC will make contact with all consumers for whom a new referral has been received to establish:
 - reason for referral
 - clinical need
 - urgency of response required.
- All phone calls/referrals will be discussed at the next multidisciplinary YCATT Intake meeting or if urgent response required discussed with the Consultant Psychiatrist immediately.
- Possible outcomes for new referrals include:

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- further triage required.
- acceptance for YCATT management and subsequent allocation of a YCATT Care Coordinator
- acute response in the community
- immediate admission for inpatient bed
- referral not accepted – these will be discussed with the referrer and recommendations made regarding alternative options including facilitation of referrals to external agencies.
- referral to Department of Child Protection and Family Support if there are immediate concerns regarding risk of physical, sexual or emotional abuse/neglect.

4.4. Clinical handover/documentation

- The FOC will:
 - Ensure there is clinical handover (iSoBAR) to the Care Coordinator/Medical Practitioner to complete transfer of clinical information and action for appropriate follow up.
 - Ensure comprehensive documentation of the clinical interaction, document all contacts with the consumer, assessments, management/safety plans and the outcome of actions taken in webPSOLIS/PSOLIS and the consumer's medical record.

5. Compliance/performance monitoring

Compliance with this policy will be monitored by the respective Programme Managers and Nurse Director via:

- Data on consumer contacts.
- Patient feedback and/or complaints
- Clinical incidents related to a delay in treatment reported through Datix Clinical Incident Management System

6. Related policy documents

- WA Health:
 - [State-wide standardised Clinical Documentation](#)
 - [Clinical Incident Management](#)

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- [Safety Planning for Mental Health Consumers](#)
- SMHS:
 - Care Coordination in mental health (SMHS: 13)
 - Community/Home visiting (SMHS: 131)
 - Acute response to mental health emergencies/crises (SMHS: 17)
 - Triage assessment of referrals in community mental health (SMHS: 14)
- FSFHG:
 - Clinical handover (FSFH-HW-POL-0035)
 - Clinical documentation (FSFH-HW-POL-0039)
 - Clinical risk assessment and management (FSFH-HW-POL-0017)

7. Related standards

- NSQHS standards:
 - Clinical Governance
 - Partnering with consumers
 - Communication for safety
 - Recognising and responding to acute deterioration
- Chief Psychiatrist Standard for Clinical Care
 - Assessment

8. Authorisation

EXECUTIVE SPONSOR: Service Director – Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	05/2017	CNS, CI	FHMHS Clinical Governance Committee	Policy Committee	05/2020
2	12/2020	CNS, CI		FSFHG Policy Committee	12/2024
3	08/2021	CNS, CI	NMPGC	Service 5 SQR Committee	12/2024
4	12/2024	CNS, CI		Service 5 SQR Committee	12/2029
5	01/2026	FSFHG Policy Team Minor template update only		FSFHG Policy Committee	12/2029

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9. Appendix

9.1. CTT business hours

Community Treatment Team	FOC hours
Fremantle CTT	08:30 - 16:00hrs
Melville CTT	08:30 - 16:00hrs
Cockburn CTT	08:30 - 16:00hrs
Assertive CTT	08:30 - 16:00hrs
Early Intervention Psychosis	08:30 - 16:00hrs
Adult Community Assessment & Treatment Team	08:30 – 16:30hrs
Youth Community Assessment & Treatment Team	08:30 – 16:30hrs
YouthReach South	08:30 – 16:30hrs
Kara Maar	08:00 – 16:30hrs